

Primary APS : Idiopathic
 Secondary APS : SLE

APS Diagnostic Criteria 1 Clinical + 1 antibody	
Requirement	Explanation
Clinical criterion	Thrombosis OR pregnancy morbidity
Laboratory criterion	Antiphospholipid antibodies
Persistence	Positive ≥12 weeks apart
⚠ Asymptomatic antibody-positive patients DO NOT meet APS criteria.	

• Persistence is required because it might be ⊕ due to infection or in a healthy individual

Diagnosis

CBC + Coagulation panel

Thrombocytopenia
 +/- cytopenia

↑ PTT (Indicate presence
 of Lupus Anticoagulant)

Mixing study
 with normal plasma

↑ PTT
 Does not
 correct with
 mixing study

Order antibodies

Lupus Anticoagulant ⇒ ↑ PTT in vitro BUT procoagulatory in vivo

Anti Cardiolipin ⇒ False positive RPR/VDRL (Syphilis)

Anti B₂ glycoprotein I ⇒ Prothrombotic

Repeat testing after 12 weeks

Positive?

Confirm
 diagnosis

Antiphospholipid Syndrome

formation of Antiphospholipid antibodies

form complexes
 with anticoagulant
 proteins

eg. C1S, Antithrombin III

Activate
 platelets
 and
 Endothelium

Hypercoagulable state

Recurrent
 Thrombosis

Recurrent
 pregnancy loss

≥3
 miscarriages
 (<10 weeks)

Unexplained
 fetal death
 (>10 weeks)

Premature
 birth
 due to placental insufficiency

- Venous
 - Deep vein thrombosis
 - Pulmonary embolism
 - Livedo reticularis
 - Ulceration (see "Chronic venous insufficiency")
- Arterial
 - Stroke, transient ischemic attacks
 - Occlusion of organ arteries (e.g., myocardial infarction)
 - Occlusion of distal extremity arteries (ischemia and gangrene)
- Capillaries: splinter hemorrhages

Treatment

Acute thrombosis:

- Heparin → Warfarin (long-term)

Pregnancy:

- Heparin + Low-dose aspirin
- Avoid warfarin (teratogenic)